Elective care performance: Oxford University Hospitals NHS Foundation Trust, August 2026

Independent analysis of NHS England RTT data, Sep 2025 – Aug 2026. Not affiliated with the trust.

Prepared by Nabil Nawaz Khan · 10 November 2026

Dashboard, SQL code, data dictionary and cleaning log:
github.com/NabilNawazKhan/oxford-elective-performance

At a glance

Measure	August 2026	Change since Sep 2025	Benchmark
Waiting under 18 weeks	62.2%	+2.5 points	England average 65.2%; target 70% by March 2027; standard 92%
Waiting list (pathways)	89,879	+5.3%	Lists expected to fall at every trust in 2026/27
Waits over 52 weeks	1,516	−39%	1.7% of the list
Median wait	13.3 weeks	—	England 12.2 weeks

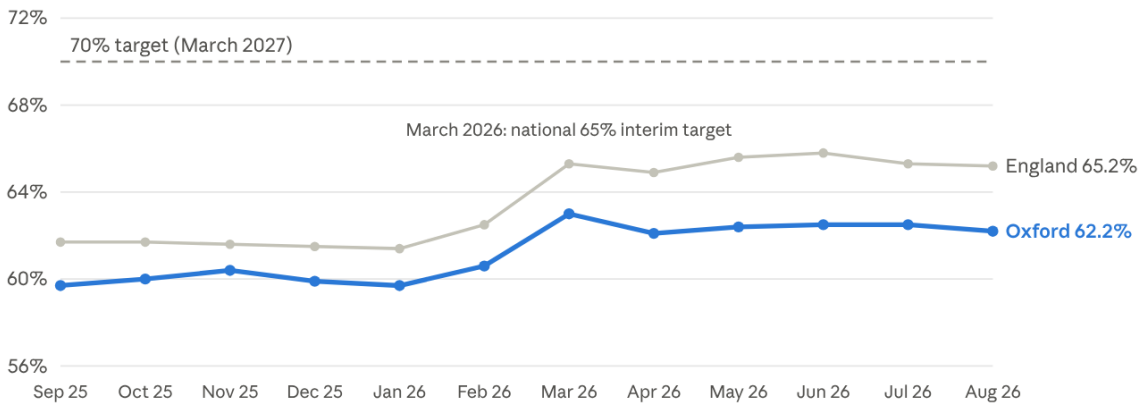
Key messages

1. **Oxford is below the England average, and the gap is widening.** The gap in 18-week performance grew from 2.0 to 3.0 percentage points over the year (2.1 to 3.0 against the rest of England, excluding Oxford).
2. **The March 2027 target is unlikely to be met on current progress.** At the past year's rate of improvement (about 0.2 points a month), Oxford would reach about 64% by March 2027, against a 70% target.
3. **Long waits have fallen, but are concentrated.** Waits over 52 weeks fell 39%. Gynaecology now holds 22% of them (339, up from 260), and is the only specialty where they rose substantially.
4. **The list is not falling as expected.** More pathways were added than completed in every month. The list has edged down since its May 2026 peak (91,088), but is still 2.0% larger than at the end of March 2026.

18-week performance

Oxford is 3.0 points below the England average on 18-week performance

% of waiting-list pathways under 18 weeks, Sep 25 to Aug 26. Gap in Aug 26: 3.0 points



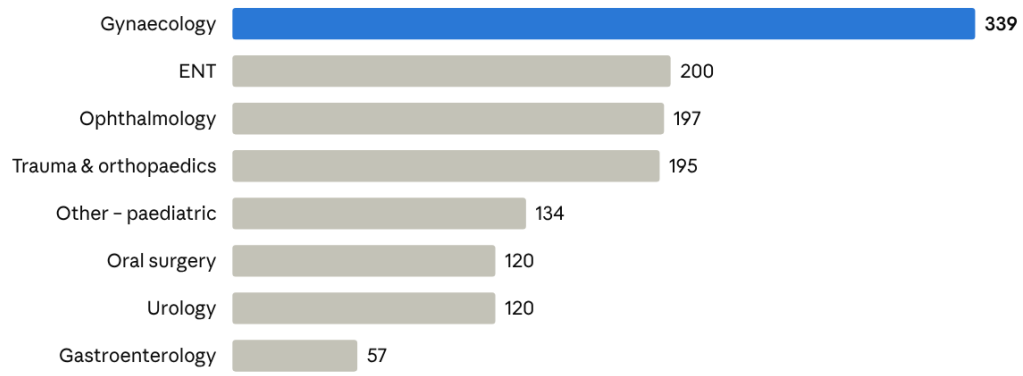
NHS England RTT full extracts, Sep 2025 – Aug 2026. Oxford excludes non-English commissioners; England uses submitted data, without estimates for trusts that did not submit.

Both Oxford and England jumped in March 2026, the deadline for the national 65% interim target. Nationally, the rise was reportedly helped by £120 million of "sprint funding" (Full Fact). Since then, England has held its gain, while Oxford has slipped back slightly.

Specialties

Gynaecology holds 22% of Oxford's waits over 52 weeks

Waits over 52 weeks, August 2026: the 8 specialties with the most. Gynaecology = 22% of 1,516 across the trust
Gynaecology is the only specialty where these waits rose substantially (from 260 in September 2025)



NHS England RTT full extract, August 2026; matches NHS England's Incomplete Provider file for Oxford. Excludes non-English commissioners.

Over the same period, Gynaecology's 18-week performance improved (53.5% → 61.2%). This suggests newer referrals are being seen faster, while the longest waiters are not.

Where 18-week performance fell, September 2025 → August 2026

Specialty	Waiting (Aug 2026)	Sep 2025	Aug 2026	Change (points)
Plastic surgery	1,906	66.5%	59.5%	−7.0
Urology	4,653	59.1%	54.0%	−5.1
Elderly medicine	628	95.1%	90.4%	−4.7
Gastroenterology	3,680	70.5%	67.0%	−3.5
ENT	7,257	52.2%	48.7%	−3.4
General surgery	1,631	71.9%	68.6%	−3.3
Other – surgical	5,947	67.8%	65.4%	−2.4
Ophthalmology	5,430	52.3%	50.8%	−1.5

Changes use unrounded figures, so they can differ by 0.1 from the rounded percentages shown (as for ENT).

How the list moved, and the outlook

October 2025 – August 2026	Pathways
Waiting list, end of September 2025	85,326
+ New pathways added (clock starts)	229,356
− Completed (treatment started, or discharged)	197,266
− Left the list without a reported completion	27,537
Waiting list, end of August 2026	**89,879**

Completions covered 86% of new pathways. Without the 27,537 pathways that left the list unreported, it would have grown by 32,090 rather than 4,553. The data does not say why these pathways left; NHS England calls them "unreported removals", and list validation, which the national planning framework asks trusts to carry out, is one possible reason.

Outlook to August 2027 (straight-line projections):

- **About 96,000** if the last six months' trend continues (+491 a month)
- **About 85,000** if the last three months' trend continues (−403 a month)
- **About 74,000** with 10% more completed pathways (about 1,800 more a month)

The planning framework expects waiting lists to fall at every trust in 2026/27. Oxford's list is 2.0% larger than at the end of March 2026.

Recommendations

1. **Gynaecology long waits.** Review the 339 pathways waiting over 52 weeks, including the 18 over 65 weeks, and report progress monthly.
2. **Specialties that are slipping.** Investigate why 18-week performance fell in Plastic surgery, Urology and ENT (demand, capacity and referral routes), and add specialty-level 18-week trends to monthly reporting.
3. **Capacity.** Plan for about 1,800 more completed pathways a month (+10%), alongside demand management such as Advice and Guidance, which the planning framework prioritises.

Data and limitations

- **Source:** NHS England consultant-led RTT waiting times, September 2025 – August 2026 (Open Government Licence v3.0). 2.18 million rows were processed in SQL Server, with the dashboard built in Power BI. Every step, check and data issue is logged in the GitHub repository above.
- **Validation:** Oxford figures exclude non-English commissioners, and match NHS England's August 2026 provider file exactly (89,879 pathways; 62.2% within 18 weeks; 1,516 over 52 weeks). The median wait (13.3 weeks) is taken from that file.
- **England:** percentages use submitted data, without estimates for trusts that did not submit, and in the four months checked against NHS England's published figures (March, June, July and August 2026) they matched or differed by 0.1 points. The England median (12.2 weeks) is from NHS England's August 2026 statistical press notice. England figures include Oxford, which is 1.3% of England's waiting list; excluding Oxford changes England's figure by at most 0.1 points in any month.
- **Forecast:** straight-line projections, with no seasonal patterns, assuming removals continue at recent rates.
- **Target:** the trust-level target ("a minimum 7% improvement in 18-week performance or a minimum of 65%, whichever is greater") does not say whether 7% means percentage points or a relative change, or which month is the baseline, so the 70% national target is used.
- **Counts** are pathways, not patients: some patients wait on more than one pathway.